

Start *Strong*

FERTILITY CHECKLIST

Everything to do in the three months before you start trying for a baby, one step at a time.

3 months before | 1 month before | When you start trying

WELCOME

A strong start begins *before* the positive test

The months before you conceive shape the earliest weeks of pregnancy, often before you even know you're pregnant. The good news: most of what matters is simple, and you can do it at your own pace. This checklist walks you through all of it.

3 months before

Checkups, tests, supplements, and habits. The biggest part of your prep.

1 month before

Stop birth control, start tracking, and double-check the essentials.

When you start trying

Know your fertile days, keep it relaxed, and know when to ask for help.

How to use this checklist

- ◯ **Go through it with a pen**
  Tick things off as you go. There's a one-page master checklist at the end you can print and stick on the fridge.
- ◯ **Don't aim for perfect**
  Missed something? Start where you are. Every item you tick helps, even if you're already trying.
- ◯ **Bring it to your doctor**
  Pages 4–8 are built to take to your preconception visit, so nothing important gets forgotten.
- ◯ **Track it in Mia**
  Track your cycle in the Mia app and find more helpful information about preparing for pregnancy along the way.

Planning with a partner? Their health matters too. Sperm take about 2.5 months to develop, so the three-month timeline applies to both of you.

YOUR PLAN AT A GLANCE

Three months, five *simple* steps

Here's the whole plan on one page. Each step has its own section with details and checkboxes.

STEP 1 · 3 MONTHS BEFORE

Get a health checkup

Book a preconception visit, get the right tests, update vaccines, and review your medications.

STEP 2 · 3 MONTHS BEFORE

Nourish your body

Start folic acid and a prenatal vitamin, and learn which foods to enjoy and which to skip.

STEP 3 · ONGOING

Build healthy habits

Alcohol, smoking, caffeine, movement, sleep, and a safer home and workplace.

STEP 4 · ONGOING

Get your life ready

Insurance, parental leave, budget, and your support network.

STEP 5 · 1 MONTH BEFORE AND BEYOND

Get ready to try

Stop birth control, start tracking your cycle, and learn your best days to conceive.

Book your *preconception* visit

A visit to your OB-GYN or family doctor before you start trying is the single most useful step on this list. It's where hidden issues get caught early, while there's still time to fix them.

Before the appointment

- ◯ **Book it about 3 months ahead**
  That leaves time for tests, vaccines, and any treatment before you start trying.
- ◯ **Write down your cycle details**
  First day of your last period, typical cycle length, anything unusual.
- ◯ **List every medication**
  Prescriptions, over-the-counter pills, supplements, and herbal products.
- ◯ **Ask family about health history**
  Both sides: genetic conditions, birth defects, diabetes, blood clots, early pregnancy loss.
- ◯ **Find your vaccination records**
  Especially MMR, chickenpox, and hepatitis B.

Questions to ask

- ◯ **Which tests do I need?**
  See the full lists on the next three pages.
- ◯ **Are my medications safe for pregnancy?**
  Never stop a prescription on your own. Ask first.
- ◯ **Should I have carrier screening?**
  A blood test that checks for genetic conditions you could pass on.
- ◯ **Do I need a higher dose of folic acid?**
  Some people do, for example if you take seizure medication or had a pregnancy affected by a neural tube defect.
- ◯ **How long should we try before coming back?**
  It depends on your age and health history.

Have a health condition? Diabetes, thyroid disease, high blood pressure, epilepsy, or depression don't rule out pregnancy, but they're best brought under good control before you conceive. Ask your doctor to make a plan with you.

Tests *most people* should have

These are the standard checks at a preconception visit. Your doctor may already have some results on file. If so, just make sure they're recent.

Your baseline

- ◯ **Blood pressure, weight, and BMI**
  A starting point for your pregnancy.
- ◯ **Complete blood count**
  Checks for anemia and can hint at inherited blood conditions like thalassemia.
- ◯ **Blood type, Rh factor, and antibody screen**
  If you're Rh-negative, it's good to know before pregnancy.
- ◯ **Rubella immunity**
  Shows whether you need the MMR vaccine before trying.
- ◯ **Chickenpox immunity**
  If you're not sure you've had chickenpox or the vaccine.
- ◯ **Pap and HPV test**
  Make sure your cervical screening is up to date.

Infections and more

- ◯ **HIV**
  Recommended for everyone planning a pregnancy.
- ◯ **Hepatitis B**
  Also shows whether you're protected or need the vaccine.
- ◯ **Hepatitis C**
  Recommended at least once for all adults.
- ◯ **Syphilis**
  Rates are rising, and it's easily treated.
- ◯ **Chlamydia and gonorrhea**
  Often have no symptoms but can affect fertility and pregnancy.
- ◯ **Dental checkup**
  Gum disease is easier to treat now, and X-rays are simpler before pregnancy.



Results to keep

Save copies of all your results in one folder. You'll use them again at your first prenatal appointment, and they help any new doctor get up to speed quickly.

Your genes and *family history*

Some conditions run in families even when no one has symptoms. Checking before pregnancy gives you the most time and the most options.

Map your family history

- **Ask about both sides**
  Parents, siblings, grandparents, aunts, uncles, and cousins, for you and your partner.
- **Genetic conditions and birth defects**
  Such as cystic fibrosis, sickle cell disease, heart defects, or spina bifida.
- **Intellectual disability or developmental delays**
  Including fragile X syndrome.
- **Pregnancy losses**
  Repeated miscarriages, stillbirths, or babies who died young.
- **Early menopause, blood clots, early heart disease**
  Early menopause in the family can point to fragile X carrier status.
- **Your ancestry**
  Some conditions are more common in certain backgrounds.

Carrier screening

FOR EVERYONE

Cystic fibrosis and spinal muscular atrophy
Offered to everyone planning a pregnancy.

BASED ON BLOOD COUNT OR ANCESTRY

Sickle cell disease and thalassemia
More common with African, Mediterranean, Middle Eastern, or Asian ancestry.

BASED ON ANCESTRY

Tay-Sachs and related conditions
For Ashkenazi Jewish, French-Canadian, or Cajun ancestry.

BASED ON FAMILY HISTORY

Fragile X syndrome
If a relative has fragile X, intellectual disability, or early menopause.

You can also ask about an expanded panel that checks for hundreds of conditions at once.

If you're both carriers

Of the same condition, each pregnancy has:



25% affected — 50% carrier — 25% not a carrier

See a genetic counselor if

- You'll be 35 or older at delivery
- You're both carriers of the same condition
- You've had 2 or more miscarriages
- You and your partner are related by blood

Tests you *may* need

These aren't needed for everyone. Tick the ones where the reason fits you, and bring them up with your doctor.

- ◯ **Ferritin (iron stores)**
  If you have heavy periods, eat plant-based, or have had anemia.
- ◯ **Thyroid (TSH)**
  If you have symptoms, a thyroid history, or irregular cycles.
- ◯ **Blood sugar or HbA1c**
  If you have PCOS, a higher BMI, a family history of diabetes, or had gestational diabetes.
- ◯ **Vitamin D**
  If you get little sun, have darker skin, or have a condition that affects absorption.
- ◯ **Vitamin B12**
  If you're vegan, take metformin, or have had stomach surgery.
- ◯ **Kidney and liver tests**
  If you have a chronic condition or take long-term medication.
- ◯ **Hormone tests (FSH, AMH, prolactin, testosterone)**
  If your periods are irregular or absent, or you're 35 or older.
- ◯ **Pelvic ultrasound**
  If your periods are very painful or heavy, or fibroids or endometriosis are suspected.
- ◯ **Tuberculosis screening**
  If you've lived in or traveled to a high-risk area, or were exposed.
- ◯ **Toxoplasmosis and CMV antibodies**
  Not routine in the U.S. Ask if you work with young children or have cats.
- ◯ **Zika testing and travel advice**
  If you or your partner have traveled to an area with Zika.

More tests aren't always better. A good question for any test: "Will this result change what we do?" Your doctor can help you focus on the ones that matter for you.

Vaccines and *medications*

Some vaccines can't be given during pregnancy, and some medications need to be switched before you conceive. Both take a little planning.

Vaccines

- ◯ **MMR (measles, mumps, rubella), if you're not immune**
  Get it early in your plan: wait at least 1 month afterward before trying.
- ◯ **Chickenpox (varicella), if you're not immune**
  Also wait at least 1 month afterward before trying.
- ◯ **HPV, if you haven't finished the series**
  It isn't given during pregnancy, so finish it beforehand if you're eligible.
- ◯ **Hepatitis B, if you're not protected**
  Can be given before or during pregnancy.
- ◯ **Tdap, if you're not up to date**
  You'll also get a dose during each pregnancy to protect your newborn.
- ◯ **Flu, COVID-19, and other seasonal vaccines**
  Recommendations can change from year to year, so ask what's advised for you.

Medications and skincare

- ◯ **Review every prescription with your doctor**
  Some acne, seizure, blood pressure, and blood-thinning medications aren't safe in pregnancy.
- ◯ **Check over-the-counter medicines, herbs, and teas**
  “Natural” doesn't always mean safe in pregnancy.
- ◯ **Check your skincare**
  Ask about retinoid creams and serums, like retinol or tretinoin.
- ◯ **Don't stop anything on your own**
  Stopping some medicines suddenly can be riskier than continuing them.

Supplements that *actually* matter

You don't need a cabinet full of pills. A good prenatal vitamin covers most of what you need. These are the nutrients to check for on the label.

400 mcg a day

Folic acid

Lowers the risk of neural tube defects, which form in the first weeks, often before you know you're pregnant. Start at least **1 month before** trying.

150 mcg a day

Iodine

Supports your thyroid and your baby's brain development. Not every prenatal vitamin includes it, so check the label.

Test first

Vitamin D

Supports bones and immunity. Your doctor can recommend a dose based on your blood test.

DHA omega-3

Omega-3 fats

Important for your baby's brain and eyes. Eat low-mercury fish, or ask your doctor about a supplement.

- ◯ **Start a prenatal vitamin with folic acid**
  Take it every day. Pairing it with something you already do, like brushing your teeth, makes it easier to remember.
- ◯ **Check it contains iodine**
  Look for about 150 mcg per daily dose.
- ◯ **Ask about iron and vitamin D**
  Only take extra if your tests show you need it.
- ◯ **Skip high-dose vitamin A**
  Too much preformed vitamin A (retinol) can harm a developing baby.

Eat well, and eat *safely*

There's no special fertility diet you have to follow. Balanced, varied meals do the job. It's also a good time to learn the food-safety habits you'll need in pregnancy.

Fill your plate with

- Vegetables and fruit at most meals
- Whole grains, beans, and lentils
- Lean protein, eggs, and nuts
- Dairy or fortified alternatives for calcium
- 2–3 servings a week of low-mercury fish, like salmon, sardines, shrimp, or cod

Start cutting back on

- High-mercury fish: shark, swordfish, king mackerel, tilefish, bigeye tuna, marlin, orange roughy
- Unpasteurized milk, juice, and soft cheeses made from raw milk
- Raw or undercooked meat, eggs, and seafood
- Cold deli meats unless heated until steaming

- ◯ **Wash fruits and vegetables well**
  Soil can carry toxoplasmosis and other germs.
- ◯ **Cook meat all the way through**
  Use a thermometer if you're unsure.
- ◯ **Hand off the cat litter**
  Or wear gloves and wash your hands after. Cat feces can spread toxoplasmosis.
- ◯ **Aim for a healthy weight, gently**
  Being underweight or overweight can affect ovulation. Small, steady changes are enough.

Vegetarian or vegan? You can have a healthy pregnancy on a plant-based diet. Ask your doctor about vitamin B12, iron, iodine, and omega-3s, which can be harder to get from plants alone.

Small habits, *big* difference

You don't need to overhaul your life. Focus on the few habits with the strongest effect on fertility and early pregnancy.

0
drinks of alcohol once you're trying. No amount is known to be safe in pregnancy.

<200 mg
caffeine a day, about two 8-oz cups of brewed coffee.

150 min
of moderate movement a week, like brisk walking or swimming.

- [ ] **Stop smoking and vaping**
  Smoking harms eggs and lowers fertility. Ask your doctor about support to quit. It works better than willpower alone.
- [ ] **Stop drinking alcohol when you start trying**
  You could be pregnant for weeks before you know it.
- [ ] **Avoid cannabis and recreational drugs**
  They can affect both fertility and pregnancy.
- [ ] **Keep caffeine under 200 mg a day**
  Remember tea, energy drinks, soda, and chocolate count too.
- [ ] **Move most days**
  Keep doing what you enjoy. Regular exercise is safe while trying.
- [ ] **Protect your sleep**
  Aim for 7–9 hours. Regular sleep supports hormone balance.

One change a week

Trying to fix everything at once rarely sticks. Pick one habit this week, then add the next. In three months, you'll have covered the whole list.

A safer home, a *calmer* mind

A few everyday things at home and at work are worth checking, and so is how you're feeling. Your mental health is part of your prep, too.

Home and work

- ◯ **Check your workplace for chemicals**
  Solvents, pesticides, radiation, or heavy metals? Ask about protection or a change of tasks.
- ◯ **Use gentle cleaning products**
  Open windows and wear gloves with strong chemicals.
- ◯ **Check for lead if your home is older**
  Old paint and pipes can contain lead.
- ◯ **Skip hot tubs and saunas once trying**
  High body heat in early pregnancy can be harmful.

Mind and mood

- ◯ **Check in with yourself**
  Anxious, low, or overwhelmed? You're not alone, and it's worth talking about.
- ◯ **Tell your doctor about mental health history**
  Some conditions and medicines need a plan before pregnancy.
- ◯ **Pick one stress reliever**
  A daily walk, breathing exercise, journaling, or time with friends.
- ◯ **Talk openly with your partner**
  About timing, fears, and what support looks like for each of you.
- ◯ **Make sure you feel safe at home**
  If someone hurts, threatens, or controls you, your doctor can connect you with support.

Stress won't stop you from getting pregnant, but it can make the journey harder. Our **Protect Your Connection** PDF guide has tools to help you both stay close while trying.

When to reach out

If you've felt down, hopeless, or lost interest in things you usually enjoy for more than two weeks, talk to your doctor or a mental health professional. Getting support before pregnancy is a strong start, too.

Get your life *ready*, too

The practical side is easy to put off, but sorting it out now takes a lot of pressure off later. Tick off what applies to you.

Money and insurance

- ◯ **Check your health insurance**
  What's covered for prenatal care, birth, and fertility support?
- ◯ **Estimate pregnancy and birth costs**
  Deductibles, copays, and out-of-pocket maximums.
- ◯ **Start a baby fund**
  Even a small monthly amount adds up over nine months.
- ◯ **Review life insurance and your will**
  Not fun, but a smart step before a family grows.

Work and support

- ◯ **Read your parental leave policy**
  How much leave, paid or unpaid, and when to tell your employer.
- ◯ **Check your partner's leave, too**
  Plan how you'll share the first weeks.
- ◯ **Think about childcare**
  In some areas, waitlists open long before a baby arrives.
- ◯ **Name your support people**
  Who can you lean on for help and for honest talks?
- ◯ **Choose your prenatal care provider**
  An OB-GYN or midwife, and where you'd like to give birth.



You don't need everything figured out

No one is ever fully “ready” for a baby. The goal here is to remove the biggest worries, not to plan every detail.

Money talk starters for the two of you

- What do we want our first year with a baby to look like?
- Will one of us work less, and for how long?
- What can we cut back on now to build a cushion?

One month *to go*

You're almost there. This month is about getting your cycle back on its own rhythm and making sure the essentials are in place.

When fertility returns after birth control

Pill, patch, or ring	Usually within 1–3 months
Hormonal or copper IUD	Soon after removal
Implant	Soon after removal
Injection (Depo-Provera)	Can take 10 months or more, so plan ahead
Condoms or other barrier methods	Right away

- ◯ **Taking folic acid for at least a month**
  If not, start today. It's never too late to begin.
- ◯ **Stop birth control when you're ready**
  It's fine to start trying straight away. There's no need to wait for a “clean” cycle.
- ◯ **Start tracking your cycle in Mia**
  Log the first day of each period. Your cycle length tells you when you're likely to ovulate.
- ◯ **Finish any vaccines**
  Remember the one-month wait after MMR and chickenpox vaccines.
- ◯ **Stock up on the basics**
  Prenatal vitamins, ovulation tests if you want them, and pregnancy tests.

No period 3 months after stopping birth control, and a negative pregnancy test? Check with your doctor. It's usually nothing serious, but it's worth finding out why.

When you start *trying*

This is the exciting part. A few simple facts help you make the most of every cycle, without turning it into a job.

6
fertile days per cycle: the 5 days before ovulation plus ovulation day

1–2
days between tries during your fertile window is plenty

~14
days before your next period is when ovulation usually happens

- ◯ **Find your fertile window**
  Subtract 14 from your usual cycle length to estimate your ovulation day, then count back 5 days.
- ◯ **Watch for body signs**
  Clear, stretchy discharge and a positive ovulation test mean ovulation is close.
- ◯ **Keep sex fun, not scheduled to the minute**
  Every 1–2 days in your window is enough. Exact timing doesn't improve your chances.
- ◯ **Test on the day your period is due**
  Testing earlier often gives a false negative.

When to ask your doctor for help

Under 35
after 12 months of trying

35 to 39
after 6 months of trying

40 and over
right away, when you start

See a doctor sooner if your periods are irregular or absent, or if you have a known condition like PCOS or endometriosis.

Got a positive test? Congratulations! Keep taking your prenatal vitamin and call your doctor to book your first prenatal appointment.

PRINT ME

Your master *checklist*

Everything on one page. Stick it on the fridge and tick as you go.

Step 1 · Health checkup

- ◯ Book a preconception visit
- ◯ List medications and supplements
- ◯ Map your family health history
- ◯ Blood count, blood type, and Rh factor
- ◯ Rubella and chickenpox immunity
- ◯ HIV, hepatitis B and C, syphilis
- ◯ Chlamydia, gonorrhea, Pap/HPV
- ◯ Carrier screening (CF, SMA, and more)
- ◯ Ask which extra tests fit you
- ◯ Update vaccines (MMR, chickenpox, HPV, Tdap, flu)
- ◯ Review prescriptions and skincare
- ◯ Dental checkup

Step 2 · Nourish

- ◯ Start a prenatal vitamin with folic acid
- ◯ Check it has iodine
- ◯ Eat low-mercury fish 2–3 times a week
- ◯ Cut high-mercury fish and raw foods
- ◯ Hand off the cat litter

Step 3 · Habits

- ◯ Quit smoking and vaping
- ◯ Stop alcohol when trying
- ◯ Caffeine under 200 mg a day
- ◯ Move 150 minutes a week
- ◯ Sleep 7–9 hours
- ◯ Check work and home for chemicals
- ◯ Pick one daily stress reliever

Step 4 · Life ready

- ◯ Check health insurance coverage
- ◯ Estimate costs and start a baby fund
- ◯ Read parental leave policies
- ◯ Name your support people

Step 5 · Ready to try

- ◯ Folic acid for at least 1 month
- ◯ Stop birth control
- ◯ Track your cycle in Mia
- ◯ Stock up on ovulation and pregnancy tests
- ◯ Know your fertile window

NOTES

Notes for your *next visit*

Keep going with Mia

Keep tracking your cycle in the Mia app and explore more helpful information about getting ready for pregnancy. For your partner's side of the prep, see the **Maximize Your Chances** PDF guide, and for staying close as a couple, the **Protect Your Connection** PDF guide.

Sources

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This checklist is for general education and isn't medical advice. Talk to your doctor about what's right for you. Made with care by the team behind Mia.